

Medicaid coverage of obesity drugs: what it did and what it could cost

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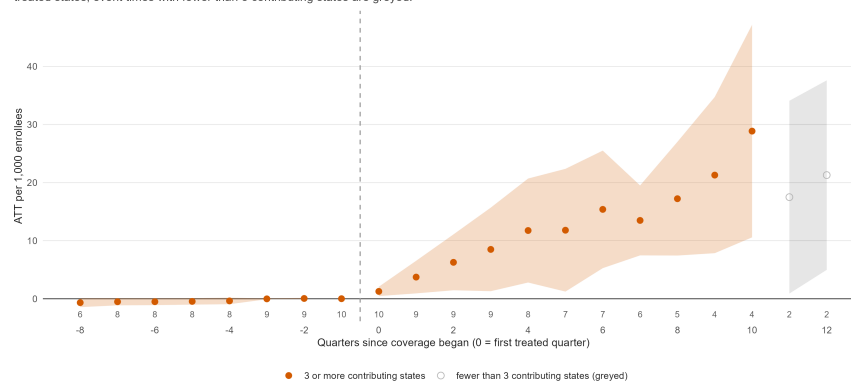
Question. What did Medicaid coverage of Wegovy and Zepbound do to prescriptions, how many adults are eligible, and what could coverage cost a program of 1 million enrollees over five years?

Three findings

1. **Coverage was associated with more prescriptions.** An estimated 12.2 additional prescriptions per 1,000 enrollees per quarter (95% CI 8.4 to 16.0), rising from 1.3 in the first quarter to 17.2 after eight; ten covering states against 34 never-covering jurisdictions. A placebo shows 0.4 (95% CI -0.1 to 0.8).
2. **Many adults are eligible.** 128.8 million U.S. adults meet the FDA label criteria (lower bound; 95% CI 117.2 to 140.3 million), including 17.8 million with Medicaid (95% CI 14.7 to 20.9 million).
3. **The budget depends on the rebate.** Central five-year net cost \$161.3 million (\$2.69 per enrollee per month; rebate 51.2% assumed); scenario range \$55.6 to \$369.4 million; \$68.3 million at the announced \$245 price.

Wegovy/Zepbound prescriptions per 1,000 Medicaid enrollees rose by an estimated 1.3 (95% CI 0.5 to 2.1) at coverage start, 11.7 (95% CI 2.8 to 20.7) after 4 quarters and 17.2 (95% CI 7.4 to 27.0) after 8 quarters

Callaway-Sant'Anna dynamic ATT, 10 primary states vs never- and not-yet-treated states, 2018 Q1 to 2025 Q3; 20 imputations of suppressed cells combined with Rubin's rules. Shaded band and points: pointwise 95% CI; numbers along the bottom: contributing treated states; event times with fewer than 3 contributing states are greyed.



Simultaneous confidence bands are not shown: with single-state cohorts the bootstrap critical value is unreliable (did warn of this) and very large (17.9 on average over the imputations, versus 1.06 pointwise). Leads before 2021 Q2 (before Wegovy existed) are omitted: the outcome is mechanically zero there and they are not evidence of parallel trends. Source: CMS State Drug Utilization Data and Medicaid enrollment (warehouse marts). Gross of rebates; counts under 11 suppressed by CMS.

Figure 1: Coverage effect by quarter since coverage began (source: CMS State Drug Utilization Data and Medicaid enrollment; part of the growth is national market growth)

Methods. Public data (CMS Medicaid State Drug Utilization Data and enrollment, NHANES, MEPS, KFF polls); difference-in-differences for staggered adoption with imputed suppressed cells; survey-weighted eligibility; ISPOR-style budget impact model with probabilistic sensitivity analysis. Warehouse: PostgreSQL and dbt (54 models, 126 tests); analysis in R; Quarto and Shiny.

Skills. Real-world evidence · HEOR · market access · claims data · NDC coding · causal inference · budget impact · SQL · dbt · R · Quarto · Shiny.

Links. Interactive budget model: <https://01a108a8-ecde-397c-5353-39196812b10c.share.connect.posit.cloud/> · Code: github.com/erickyegon/incretin-access-value · Report: report/report.html · Deck: deck/deck.pdf.

Public aggregate data; no company affiliation or endorsement; not patient-level claims; gross of rebates unless stated; scenarios, not forecasts. I used AI tools to help write code and documentation. The study design, methods and conclusions are my own, and I verified all results.